

For the use of a Registered Medical Practitioner or a Hospital or Laboratory only.

FEMCORD

(Fulvestrant Solution for injection in pre-filled syringe 250 mg/5 mL)

Qualitative and quantitative composition

Each pre-filled syringe of 5 mL contains Fulvestrant USP 250 mg.

For a full list of excipients, see 'List of excipients'.

Pharmaceutical Form

Solution for injection, pre-filled syringe.

Product description: A clear, colorless to yellow, viscous solution, filled in a clear glass pre-filled syringe. When examined under suitable conditions of visibility it should be practically free from foreign particles.

Therapeutic indications

Femcor® is indicated:

- 27 months for the treatment of estrogen receptor positive, locally advanced or metastatic breast cancer in postmenopausal women:
 - who are human epidermal growth factor receptor 2 (HER2)-negative and not previously treated with endocrine therapy.
 - with disease relapse on or after adjuvant endocrine therapy, or disease progression on endocrine therapy.
- in combination with ribociclib for the treatment of hormone receptor (HR)-positive, human epidermal growth factor receptor 2 (HER2)-negative advanced or metastatic breast cancer in postmenopausal women, as initial endocrine based therapy or following disease progression on endocrine therapy.
- in combination with abemaciclib for the treatment of women with hormone receptor (HR)-positive, human epidermal growth factor receptor 2 (HER2)-negative advanced or metastatic breast cancer with disease progression following endocrine therapy.
- in combination with palbociclib for the treatment of hormone receptor (HR)-positive, human epidermal growth factor receptor 2 (HER2)-negative locally advanced or metastatic breast cancer in women who have received prior endocrine therapy (see Pharmacodynamic Properties).

In pre- or perimenopausal women, the combination treatment with palbociclib or abemaciclib should be combined with a luteinizing hormone releasing hormone (LHRH) agonist. This indication is only applicable in markets where palbociclib, abemaciclib or abemaciclib are registered.

Posology and method of administration

Posology:

Adult females (including the elderly):

The recommended dose is 500 mg at intervals of one month, with an additional 500 mg dose given two weeks after the initial dose.

When Fulvestrant is used in combination with palbociclib, abemaciclib or ribociclib, please also refer to the Summary of Product Characteristics of palbociclib, abemaciclib or ribociclib.

Prior to the start of treatment with the combination of Fulvestrant plus palbociclib or abemaciclib, and throughout the duration, premenopausal women should be treated with LHRH agonists according to local clinical practice.

Special populations

Paediatric patient:

The safety and efficacy of Fulvestrant in children from birth to 18 years of age have not been established. Currently available data are described in sections Pharmacodynamic Properties and Pharmacokinetic Properties, but no recommendation on a posology can be made.

Renal impairment:

No dose adjustments are recommended for patients with mild to moderate renal impairment (creatinine clearance ≥ 30 mL/min). Safety and efficacy have not been evaluated in patients with severe renal impairment (creatinine clearance < 30 mL/min). Therefore, caution is recommended in these patients (see Special Warnings and Precautions for Use).

Hepatic impairment:

No dose adjustments are recommended for patients with mild to moderate hepatic impairment. However, as fulvestrant exposure may be increased, Fulvestrant should be used with caution in these patients. There are no data in patients with severe hepatic impairment (see Contraindications, Special Warnings and Precautions for Use and Pharmacokinetic Properties).

Method of administration:

Fulvestrant should be administered as two consecutive 5 mL injections by slow intramuscular injection (1-2 minutes/injection), one in each buttock (gluteal area).

Caution should be taken if injecting Fulvestrant into the gluteal muscle due to the proximity of the underlying sciatic nerve.

For detailed instructions for administration, see Instructions for administration and Special precautions for disposal.

Contraindications:

- Hypersensitivity to the active substance, or to any of the other excipients.
- Pregnancy and lactation (see Pregnancy and lactation).
- Severe hepatic impairment (see Special Warnings and Precautions for Use and Pharmacokinetic Properties).

Special warnings and precautions for use:

This preparation contains benzyl alcohol, its use should be avoided in children under two years of age.

Not to be used in neonates.

Fulvestrant should be used with caution in patients with mild to moderate hepatic impairment (see Posology and Method of administration, Contraindications, and Pharmacokinetic Properties).

Fulvestrant should be used with caution in patients with severe renal impairment (creatinine clearance less than 30 mL/min) (see Pharmacokinetic Properties).

Due to the intramuscular route of administration, Fulvestrant should be used with caution if treating patients with bleeding diatheses, thrombocytopenia or those taking anticoagulant treatment.

Thrombotic/embolic events are commonly observed in women with advanced breast cancer and have been observed in clinical studies with Fulvestrant (see Undesirable Effects). This should be taken into consideration when prescribing Fulvestrant to patients at risk.

Injection site related events including scarring, neuralgia, neuropathic pain, and peripheral neuropathy have been reported with Fulvestrant injection. Caution should be taken while administering Fulvestrant at the site of the peripheral injection site due to the proximity of the underlying sciatic nerve (see Posology and Method of Administration and Undesirable Effects).

There are no long-term data on the effect of Fulvestrant on bone. Due to the mechanism of action of Fulvestrant, there is a potential risk of osteoporosis.

The efficacy and safety of Fulvestrant (either as monotherapy or in combination with palbociclib) have not been studied in patients with critical visceral disease.

When Fulvestrant is combined with palbociclib, please also refer to the Summary of Product Characteristics of palbociclib.

Interference with estradiol antibody assays:

Due to the structural similarity of Fulvestrant and estradiol, Fulvestrant may interfere with antibody based-estradiol assays and may result in falsely increased levels of estradiol.

Paediatric population:

Fulvestrant is not recommended for use in children and adolescents as safety and efficacy have not been established in this group of patients (see section Pharmacodynamic Properties).

Interactions with other medicinal products and other forms of interaction:

A clinical interaction study with midazolam (substrate of CYP 3A4) demonstrated that Fulvestrant does not inhibit CYP 3A4. Clinical interaction studies with clonidine (inducer of CYP 3A4) and ketoconazole (inhibitor of CYP 3A4) showed no clinically relevant change in Fulvestrant clearance. Dose adjustment is therefore not necessary in patients who are receiving Fulvestrant and CYP 3A4 inhibitors or inducers concomitantly.

Fertility, pregnancy and lactation:

Women of childbearing potential:

Patients of child-bearing potential should use effective contraception during treatment with Fulvestrant and for 2 years after the last dose.

Pregnancy:

Fulvestrant is contraindicated in pregnancy (see Contraindications). Fulvestrant has been shown to cross the placenta after single intramuscular doses in rat and rabbit. Studies in animals have shown reproductive toxicity including an increased incidence of foetal abnormalities and deaths (see Preclinical Safety Data). If pregnancy occurs while taking Fulvestrant, the patient must be informed of the potential hazard to the foetus and potential risk for loss of pregnancy.

Breast-feeding:

Breast-feeding must be discontinued during treatment with Fulvestrant. Fulvestrant is excreted in milk in lactating rats. It is not known whether Fulvestrant is excreted in human milk. Considering the potential for serious adverse reactions due to Fulvestrant in breast-fed infants, use during lactation is contraindicated (see Contraindications).

Fertility:

The effects of Fulvestrant on fertility in humans has not been studied.

Effects on ability to drive and use machines:

Fulvestrant has no or negligible influence on the ability to drive or use machines. However, since asthenia has been reported very commonly with Fulvestrant, caution should be observed by those patients who experience this adverse reaction when driving or operating machinery.

Undesirable effects:

Summary of the safety profile:

Monotherapy:

This section provides information based on all adverse reactions from clinical studies, post-marketing studies or spontaneous reports in the pooled dataset of Fulvestrant monotherapy, the most frequently reported adverse reactions were injection site reactions, asthenia, nausea, and increased hepatic enzymes (ALT, AST, ALP).

In Table 1, the following frequency categories for adverse drug reactions (ADRs) were calculated based on the Fulvestrant 500 mg treatment group in pooled safety analyses of studies that compared Fulvestrant 500 mg with anastrozole 1 mg. Where frequencies differ between the pooled safety analysis and Fulvestrant, the highest frequency is presented. The frequencies in Table 1 were based on all reported events, regardless of the investigator assessment of causality. The median duration of Fulvestrant 500 mg treatment across the pooled dataset (including the studies mentioned above plus PALOMA3) was 6.5 months.

Tabulated list of adverse reactions:

Adverse reactions listed below are classified according to frequency and System Organ Class (SOC). Frequency groupings are defined according to the following convention: Very common ($\geq 10\%$), Common ($\geq 1\%$ to $< 10\%$), Uncommon ($\geq 0.1\%$ to $< 1\%$), Rare ($< 0.1\%$). Within each frequency group, adverse reactions are reported in order of decreasing seriousness.

Table 1 Adverse Drug Reactions reported in patients treated with Fulvestrant monotherapy

Adverse reactions by system organ class and frequency

Infections and infestations

Blood and lymphatic system disorders

Immune system disorders

Metabolism and nutrition disorders

Nervous system disorders

Vascular disorders

Gastrointestinal disorders

Hepatobiliary disorders

General disorders

Skin and subcutaneous tissue disorders

Musculoskeletal and connective tissue disorders

Common

Reproductive system and breast disorders

General disorders

Investigations

Common

Investigations

Common

Investigations

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Mouth ulceration, Mucosal inflammation, Oral pain, Oropharyngeal discomfort, Oropharyngeal pain, Stomatitis.

Rash includes the following PTs: Rash, Rash maculo-papular, Rash pruritic, Rash erythematous, Rash papular, Dermatitis, Dermatitis acneiform, Toxic skin eruption.

In patients receiving Fulvestrant in combination with palbociclib in the PALOMA3 study, adverse reactions of any grade were reported in 290 (94.1%) patients, with Grade 3 neutropenia being reported in 200 (68.9%) patients, and Grade 4 neutropenia being reported in 40 (11.6%) patients.

In the Fulvestrant + placebo arm (n=172), neutropenia of any grade was reported in 7 (4.1%) patients. There were no reports of Grade 3 and 4 neutropenia in the Fulvestrant + placebo arm.

In patients receiving Fulvestrant in combination with palbociclib, the median time to first episode of any grade neutropenia was 15 days (range: 13-512 days) and the median duration of Grade 3 neutropenia was 16 days. Febrile neutropenia has been reported in 3 (0.3%) patients receiving Fulvestrant in combination with palbociclib.

Combination Therapy with Abemaciclib (MONALEESA-3):

The safety of Fulvestrant (500 mg) plus abemaciclib (150 mg twice daily) versus Fulvestrant plus placebo was evaluated in MONARCH 2. The data described below reflect exposure to Fulvestrant in 664 patients with HR-positive, HER2-negative advanced breast cancer who received at least one dose of Fulvestrant plus abemaciclib or placebo in MONARCH 2.

Median duration of treatment was 12 months for patients receiving Fulvestrant plus abemaciclib and 8 months for patients receiving Fulvestrant plus placebo.

Dose reductions due to an adverse reaction occurred in 43% of patients receiving Fulvestrant plus abemaciclib. Adverse reactions leading to dose reductions $\geq 25\%$ of patients were diarrhoea and neutropenia. Abemaciclib dose reduction due to diarrhoea of any grade occurred in 19% of patients receiving Fulvestrant plus abemaciclib compared to 0.4% of patients receiving Fulvestrant plus placebo. Abemaciclib dose reductions due to neutropenia of any grade occurred in 10% of patients receiving Fulvestrant plus abemaciclib compared to no patients receiving Fulvestrant plus placebo.

Permanent study treatment discontinuation due to an adverse event was reported in 9% of patients receiving Fulvestrant plus abemaciclib and in 3% of patients receiving Fulvestrant plus placebo. Adverse reactions leading to permanent discontinuation for patients receiving Fulvestrant plus abemaciclib were infection (2%), diarrhoea (1%), hepatotoxicity (1%), fatigue (1%), nausea (0.2%), abdominal pain (0.2%), acute injury injury (0.2%), and cerebral infarction (0.2%).

Deaths during treatment or during 30-day follow-up, regardless of causality, were reported in 18 cases (4%) of Fulvestrant plus abemaciclib treated patients versus 10 cases (3%) of Fulvestrant plus placebo treated patients. Causes of death for patients receiving Fulvestrant plus abemaciclib included: 7 (2%) patients died due to underlying disease, 4 (0.5%) due to sepsis, 2 (0.5%) due to pneumonia, 2 (0.5%) due to hepatotoxicity, and one (0.2%) due to cerebral infarction.

The most common adverse reactions reported ($\geq 20\%$) in the Fulvestrant plus abemaciclib arm were diarrhoea, fatigue, neutropenia, nausea, infections, abdominal pain, anaemia, leukopenia, decreased appetite, vomiting, and headache (Table 3). The most frequently reported ($\geq 5\%$) Grade 3 or 4 adverse reactions were neutropenia, diarrhoea, leukopenia, anaemia, and infections.

Table 3 Adverse Reactions $\geq 20\%$ of Patients Receiving Fulvestrant Plus Abemaciclib and $\geq 2\%$ Higher Than Fulvestrant Plus Placebo in MONARCH 2

Adverse Reactions

Fulvestrant plus Abemaciclib

Fulvestrant plus Placebo

All Grades

Grade 3 or 4

All Grades

Grade 3 or 4

All Grades

Grade 3 or 4

All Grades

Grade 3 or 4

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All Grades

Grade 3 or 4

Table 11: Efficacy Results in MONARCH 2 (Intent-to-Treat Population)		
	Fulvestrant plus Abemaciclib	Fulvestrant plus Placebo
Progression-Free Survival (Investigator Assessment)	N=446	N=223
Number of patients with an event (n, %)	222 (49.8)	157 (70.4)
Median (months, 95% CI)	16.4 (14.4, 19.3)	9.3 (7.4, 12.7)
Hazard ratio (95% CI) ¹	0.553 (0.449, 0.681)	
p-value ²	p<0.0001	
Overall Survival³		
Number of deaths (n, %)	211 (47.3)	127 (57.0)
Median OS in months (95% CI)	46.7 (39.2, 52.2)	37.3 (34.4, 43.2)
Hazard ratio (95% CI) ¹	0.757 (0.606, 0.945)	
p-value ²	p=0.0137	
Objective Response for Patients with Measurable Disease	N=316	N=164
Objective response rate ³ (n, %)	153 (48.1)	35 (21.3)
95% CI	42.6, 53.6	15.1, 27.6

Abbreviations: CI=confidence interval; OS=overall survival.
1. Stratified by disease site (visceral metastases vs. bone-only metastases vs. other) and endocrine therapy resistance (primary resistance vs. secondary resistance).
2. Data from a pre-specified interim analysis (77% of the number of events needed for the planned final analysis) with the p-value compared with the allocated alpha of 0.021.
3. Complete response = partial response.

Figure 4 Kaplan-Meier Curves of Progression-Free Survival: Fulvestrant Plus Abemaciclib versus Fulvestrant plus Placebo (MONARCH 2)

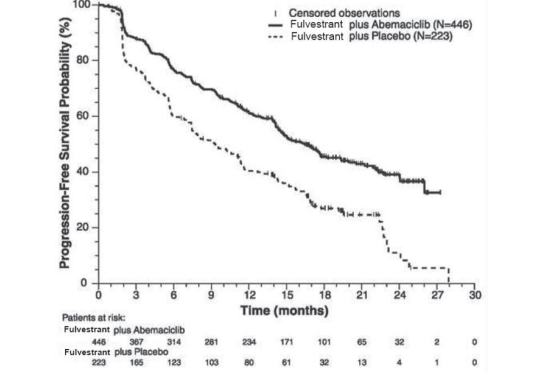
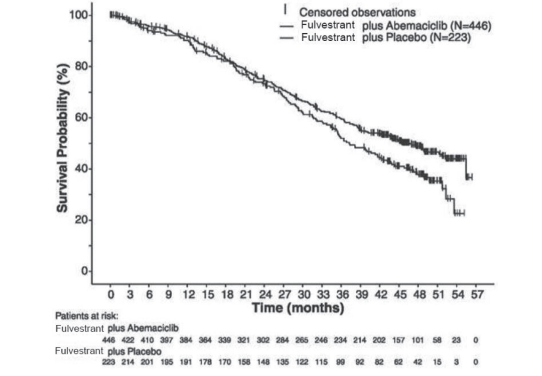


Figure 5 Kaplan-Meier Curves of Overall Survival: Fulvestrant plus Abemaciclib versus Fulvestrant plus Placebo (MONARCH 2)



Postmenopausal women with HR-positive, HER2-negative advanced or metastatic breast cancer for initial endocrine based therapy or after disease progression on endocrine therapy

Fulvestrant 500 mg in Combination with Ribociclib 600 mg (MONALEESA-3)
MONALEESA-3 (NCT 02422615) was a randomized double-blind, placebo-controlled study of Fulvestrant plus ribociclib versus Fulvestrant plus placebo conducted in postmenopausal women with hormone receptor positive, HER2-negative, advanced breast cancer who have received no or only one line of prior endocrine treatment.
A total of 726 patients were randomized in a 2:1 ratio to receive Fulvestrant plus ribociclib or Fulvestrant plus placebo and stratified according to the presence of liver and/or lung metastases and prior endocrine therapy for advanced or metastatic disease. Fulvestrant 500 mg was administered intramuscularly on Days 1, 15, 29 and once monthly thereafter, with either ribociclib 600 mg or placebo given orally once daily for 21 consecutive days followed by 7 days of until disease progression or unacceptable toxicity. The major efficacy outcome measure for the study was investigator-assessed progression-free survival (PFS) using Response Evaluation Criteria in Solid Tumors (RECIST) v1.1.
Patients enrolled in this study had a median age of 62 years (range 31 to 89). Of the patients enrolled, 47% were 65 years and older, including 14% age 75 years and older. The patients enrolled were primarily Caucasian (85%), Asian (9%), and Black (0.7%). Nearly all patients (95.7%) had an ECOG performance status of 0 or 1. First- and second-line patients were enrolled in this study (of which 19% had de novo metastatic disease). Forty-three percent (43%) of patients had received chemotherapy in the adjuvant vs. 13% in the neoadjuvant setting and 59% had received endocrine therapy in the adjuvant vs. 1% in the neoadjuvant setting prior to study entry. Twenty-one percent (21%) of patients had bone-only disease and 61% had visceral disease. Demographics and baseline disease characteristics were balanced and comparable between study arms.

The efficacy results from MONALEESA-3 are summarized in Table 12, Figure 6, and Figure 7. Consistent results were observed in stratification factor subgroups of disease site and prior endocrine treatment for advanced disease.

Table 12: Efficacy Results – MONALEESA-3 (Investigator Assessment, Intent-to-Treat Population)

	Fulvestrant plus Ribociclib	Fulvestrant plus Placebo
Progression-free survival¹	N=484	N=242
Events (n, %)	210 (43.4%)	157 (64.4%)
Median (months, 95% CI)	20.5 (18.5, 23.5)	12.8 (10.9, 16.3)
Hazard Ratio (95% CI)	0.593 (0.480 to 0.732)	
p-value ²	<0.0001	
Overall Survival	N=484	N=242
Events (n, %)	167 (34.5%)	108 (44.6%)
Median (months, 95% CI)	NR (42.5, NR)	40.0 (37.0, NR)
Hazard Ratio (95% CI)	0.724 (0.566, 0.924)	
p-value ²	0.00453	
Overall Response Rate³	N=379	N=181
	40.9 (35.9, 45.8)	28.7 (22.1, 35.3)

Abbreviation: NR, not reached.
1. p-value is obtained from the one-sided log-rank.
2. Based on confirmed responses.
3. Investigator Assessment.

Figure 6 Kaplan-Meier Progression Free Survival Curves – MONALEESA-3 (Intent-To-Treat Population, Investigator assessment)

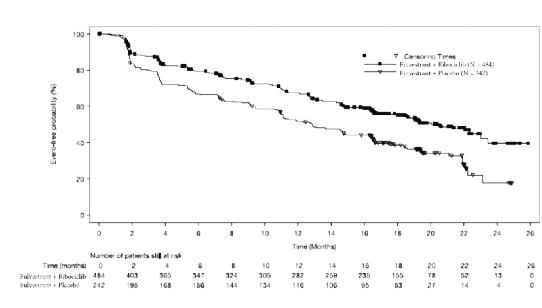
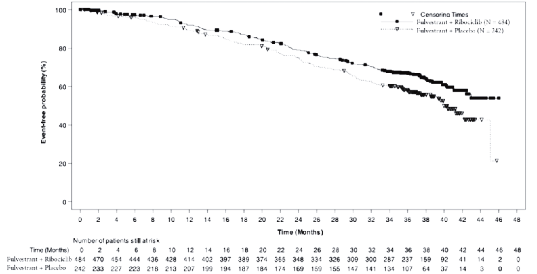


Figure 7 Kaplan-Meier plot of Overall Survival – MONALEESA-3 (Intent-to-Treat Population)



Effects on the postmenopausal endometrium
Preliminary data do not suggest a stimulatory effect of fulvestrant on the postmenopausal endometrium. A 2-week study in healthy postmenopausal volunteers treated with 20 µg per day ethinylestradiol showed that pre-treatment with fulvestrant 250 mg resulted in significantly reduced stimulation of the postmenopausal endometrium, compared to pre-treatment with placebo, as judged by ultrasound measurement of endometrium thickness.
Neoadjuvant treatment for up to 16 weeks in breast cancer patients treated with either fulvestrant 500 mg or fulvestrant 250 mg did not result in clinically significant changes in endometrial thickness, indicating a lack of agonist effect. There is no evidence of adverse endometrial effects in the breast cancer patients studied. No data are available regarding endometrial morphology.
In two short-term studies (1 and 12 weeks) in premenopausal patients with benign gynaecological disease, no significant differences in endometrial thickness were observed by ultrasound measurement between fulvestrant and placebo groups.

Effects on bone
There are no long-term data on the effect of fulvestrant on bone. Neoadjuvant treatment for up to 16 weeks in breast cancer patients with either fulvestrant 500 mg or fulvestrant 250 mg did not result in clinically significant changes in serum bone-turnover markers.

Pediatric population

Fulvestrant is not indicated for use in children.
An open-label phase II study investigated the safety, efficacy and pharmacokinetics of fulvestrant in 30 girls aged 1 to 8 years with Progressive Precocious Puberty associated with McCune Albright Syndrome (MAS). The paediatric patients received 4 mg/kg monthly intramuscular dose of fulvestrant. This 12-month study investigated a range of MAS endpoints and showed a reduction in the frequency of vaginal bleeding and a reduction in the rate of bone age advancement. The steady-state trough concentrations of fulvestrant in children in this study were consistent with that in adults (see section Pharmacokinetics/Properties). There were no new safety concerns arising from this small study, but 5-year data are yet not available.

Pharmacokinetic properties

Absorption

After administration of Fulvestrant long-acting intramuscular injection, fulvestrant is slowly absorbed and maximum plasma concentrations (C_{max}) are reached after about 5 days. Administration of Fulvestrant 500 mg regimen achieves exposure levels at, or close to, steady state within the first month of dosing (mean [CV] AUC 475 (33.4%) ng day/mL, C_{max} 25.1 (35.3%) ng/mL, C_{min} 16.3 (25.2%) ng/mL, respectively). At steady state, fulvestrant plasma concentrations are maintained within a relatively narrow range with up to an approximately 3-fold difference between maximum and trough concentrations. After intramuscular administration, the exposure is approximately dose proportional in the dose range 50 to 500 mg.

Distribution

Fulvestrant is subject to extensive and rapid distribution. The large apparent volume of distribution at steady state (V_{ss}) of approximately 3 to 5 l/kg suggests that distribution is largely extracellular. Fulvestrant is highly (95%) bound to plasma proteins. Very low density lipoprotein (VLDL), low density lipoprotein (LDL) and high density lipoprotein (HDL) fractions are the major binding components. No interaction studies were conducted on competitive protein binding. The role of sex hormone-binding globulin (SHBG) has not been determined.

Biotransformation

The metabolism of fulvestrant has not been fully evaluated, but involves combinations of a number of possible biotransformation pathways analogous to those of endogenous steroids. Identified metabolites (includes 17-ketone, sulphone, 3-sulphate, 3- and 17-glucuronide metabolites) are either less active or exhibit similar activity to fulvestrant in anti-estrogen models. Studies using human liver preparations and recombinant human enzymes indicate that CYP 3A4 is the only P450 isoenzyme involved in the oxidation of fulvestrant, however, non-P450 routes appear to be more predominant in vivo. In vitro data suggest that fulvestrant does not inhibit CYP450 isoenzymes.

Elimination

Fulvestrant is eliminated mainly in metabolised form. The major route of excretion is via the faeces, with less than 1% being excreted in the urine. Fulvestrant has a high clearance, 11±1.7 mL/min/kg, suggesting a high hepatic extraction ratio. The terminal half-life (t_{1/2}) after intramuscular administration is governed by the absorption rate and was estimated to be 50 days.

Social populations

In a population pharmacokinetic analysis of data from Phase 3 studies, no difference in fulvestrant's pharmacokinetic profile was detected with regard to age (range 33 to 89 years), weight (40-127 kg) or race.

Renal impairment

Mild to moderate impairment of renal function did not influence the pharmacokinetics of fulvestrant to any clinically relevant extent.

Hepatic impairment

The pharmacokinetics of fulvestrant has been evaluated in a single-dose clinical study conducted in women with mild to moderate hepatic impairment (Child-Pugh class A and B). A high dose of a shorter duration intramuscular injection formulation was used. There was up to about 2.5-fold increase in AUC in women with hepatic impairment compared to healthy subjects. In patients administered Fulvestrant, an increase in exposure of this magnitude is expected to be well tolerated. Women with severe hepatic impairment (Child-Pugh class C) were not evaluated.

Pediatric population

The pharmacokinetics of fulvestrant has been evaluated in a clinical study conducted in 30 girls with Progressive Precocious Puberty associated with McCune Albright Syndrome. The paediatric patients were aged 1 to 8 years and received 4 mg/kg monthly intramuscular dose of fulvestrant. The geometric mean (standard deviation) steady state trough concentration (C_{min,ss}) and AUC₀₋₂₄ was 4.2 (0.9) ng/mL and 3680 (1020) ng*hr/mL, respectively. Although the data collected were limited, the steady-state trough concentrations of fulvestrant in children appear to be consistent with those in adults.

List of excipients

10%w/v of Ethanol 96%
Benzyl alcohol
Benzyl benzoate
Refined Castor oil

Incompatibilities

In the absence of incompatibility studies, this medicinal product must not be mixed with other medicinal products.

Shelf life: 24 months.

Special precautions for storage:

Store at 2°C-8°C (in a refrigerator).
Store the pre-filled syringe in the original package in order to protect from light.

Pack size: 2 pre-filled syringes with 2 safety needles.

Instructions for administration and Special precautions for disposal and other handling
Administer the injection according to the local guidelines for performing large volume intramuscular injections.

NOTE: Due to the proximity of the underlying sciatic nerve, caution should be taken if administering Fulvestrant at the dorsogluteal injection site (see Special Warnings and Precautions for Use).

Warning - Do not autoclave safety needle (BD SafetyGlide™ Shielding Hypodermic Needle) before use. Hands must remain behind the needle at all times during use and disposal.

For each of the two syringes:

1. Remove glass syringe barrel from tray and check that it is not damaged.
2. Peel open the safety needle (SafetyGlide™) outer packaging.
3. Parenteral solutions must be inspected visually for particulate matter and discoloration prior to administration.
4. Hold the syringe upright on the ribbed part (C). With the other hand, take hold of the cap (A) and carefully tilt back and forth until the cap disconnects and can be pulled off; do not twist (see Figure 1).
5. Remove the cap (A) in a straight upward direction. To maintain sterility do not touch the syringe tip (B) (see Figure 2).
6. Attach the safety needle to the Luer-Lok and twist until firmly seated (see Figure 3).
7. Check that the needle is locked to the Luer connector before moving out of the vertical plane.
8. Pull shield straight off needle to avoid damaging needle point.
9. Transport filled syringe to point of administration.
10. Remove needle sheath.
11. Expel excess gas from the syringe.
12. Administer intramuscularly slowly (1-2 minutes/injection) into the buttock (gluteal area). For user convenience, the needle bevel-up position is oriented to the lower arm (see Figure 4).
13. After injection, immediately apply a single-finger stroke to the activation assisted lever arm to activate the shielding mechanism (see Figure 5).

NOTE: Activate away from self and others. Listen for click and visually confirm needle tip is fully covered.

Figure 1

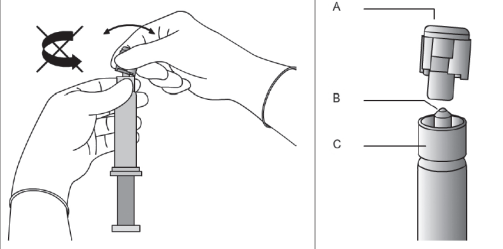


Figure 2

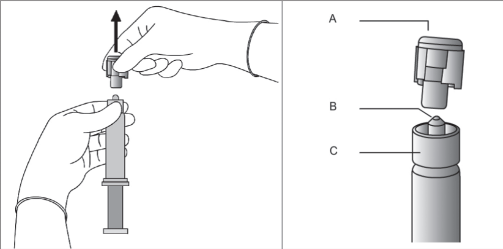


Figure 3

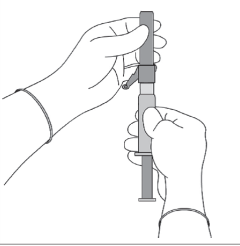


Figure 4

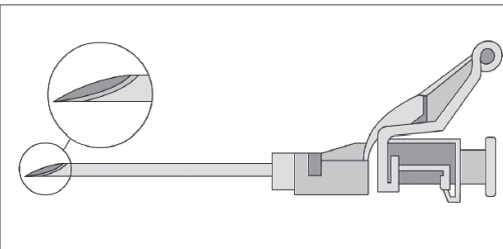
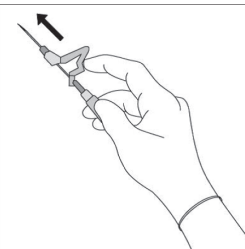


Figure 5



Disposal

Pre-filled syringes are for single use only.
Any unused medicinal product or waste material should be disposed of in accordance with local requirements.

Product Registration Holder:

Accord healthcare Sdn. Bhd.
Suite 12A-15, level 12A, Wisma Zelan, No 1 Jalan Task Permainsuri 2,
Bandar Tun Razak, 56000 Kuala Lumpur, Malaysia

Manufacturer:

Intas Pharmaceuticals Limited
Plot No 457 458, Matoda,
Plot No 191/218P, Chachanwadi,
Sarkhej Bavia Highway, Ahmedabad, 382210, India

Date of Revision of Package Insert

March 2024

RFEM012620

Back Side

RFEM012620-Femcord(Malaysia)Outsert

Size : 600 x 350 (mm), Final Folding Size : 80 x 45 (mm)

Colour : Pantone Black

Date : 25/08/23, 20/10/23, 01/03/24